

CONMY TABLETS 2mg

Terazosin hydrochloride dihydrate 2.37mg equivalent to Terazosin 2mg

Description

It is a orange, round, flat tablet, a "BH₂" is scored on one side and "S" on the other side.

Composition

Each tablet contains Terazosin hydrochloride dihydrate 2.37mg equivalent to Terazosin 2mg.

Mode of action

Benign Prostatic Hyperplasia (BPH):

The symptoms associated with BPH are related to bladder outlet obstruction, which is comprised of two underlying components: a static component and a dynamic component. The static component is a consequence of an increase in prostate size. Over time, the prostate will continue to enlarge. However, clinical studies have demonstrated that the size of the prostate does not correlate with the severity of BPH symptoms or the degree of urinary obstruction. The dynamic component is a function of an increase in smooth muscle tone in the prostate and bladder neck, leading to constriction of the bladder outlet. Smooth muscle tone is mediated by sympathetic nervous stimulation of alpha-1 adrenoceptors, which are abundant in the prostate, prostatic capsule and bladder neck. The reduction in symptoms and improvement in urine flow rates following administration of terazosin is related to relaxation of smooth muscle produced by blockade of alpha-1 adrenoceptors in the bladder neck and prostate. Because there are relatively few alpha-1 adrenoceptors in the bladder body, terazosin is able to reduce the bladder outlet obstruction without affecting bladder contractility.

Summary of pharmacodynamics and pharmacokinetics

Terazosin is rapidly and almost completely absorbed from the gastro-intestinal tract after oral administration; the bioavailability is reported to be about 90%. It is metabolized in the liver; one of the metabolites is reported to possess antihypertensive activity. The half-life in plasma is approximately 12 hours. It is excreted in faeces via the bile and in the urine, as unchanged drug and metabolites. Terazosin is 90 to 94% protein bound.

Indications

Terazosin hydrochloride is indicated for the treatment of hypertension and symptomatic benign prostatic hyperplasia (BPH)

Adverse reactions/side effects

Conmy, in common with other alpha-adrenoreceptor antagonists, may cause syncope. Syncopal episodes have occurred within 30 to 90 minutes of the initial dose of the drug. Syncope has occasionally occurred in association with rapid dosage increases or the introduction of another antihypertensive agent.

In clinical studies in hypertension, the incidence of syncopal episodes was approximately one percent. In most cases, this was believed to be due to an excessive postural hypotensive effect although occasionally the syncopal episode has been preceded by a bout of tachycardia with heart rates of 120 to 160 beats per minute.

If syncope occurs the patient should be placed in a recumbent position and given supportive treatment as necessary.

Dizziness, light-headedness or fainting may occur when standing up quickly from a lying or sitting position. Patients should be advised of this possibility and instructed to lie down if these symptoms appear and then sit for a few minutes before standing to prevent re-occurrence.

These adverse effects are self limiting and, in most cases, do not recur after the initial period of therapy or during subsequent titration.

Adverse events reported with terazosin

The most common events were asthenia, palpitations, nausea, peripheral oedema, dizziness, somnolence, nasal congestion/rhinitis and blurred vision/amblyopia.

In addition, the following have been reported: back pain; headache; tachycardia; postural hypotension; syncope; oedema; weight gain; pain in extremities; decreased libido; depression; nervousness; paraesthesia; vertigo; dyspnoea; sinusitis and impotence.

Additional adverse reactions reported in clinical trials or reported during marketing experience but not clearly associated with the use of terazosin include the following: chest pain; facial oedema; fever; abdominal pain; neck pain; shoulder pain; vasodilation; arrhythmia; constipation; diarrhoea; dry mouth; dyspepsia; flatulence; vomiting; gout; arthralgia; arthritis; joint disorders; myalgia; anxiety; insomnia; bronchitis; epistaxis; flu symptoms; pharyngitis; rhinitis; cold symptoms; pruritis; rash; increased cough; sweating; abnormal vision; conjunctivitis; tinnitus; urinary frequency; urinary tract infection and urinary incontinence primarily reported in post-menopausal women.

At least two cases of severe anaphylactoid reactions have been reported with the administration of terazosin.

Post marketing experience: Thrombocytopenia and priapism have been reported. Atrial fibrillation has been reported: however, a cause and effect relationship has not been established.

Laboratory tests: Small but statistically significant decreases in haematocrit, haemoglobin, white blood cells, total protein and albumin were observed in controlled clinical trials. These laboratory findings suggest the possibility of haemodilution. Treatment with terazosin for up to 24 months had no significant effect on prostate specific antigen (PSA) levels.

Warning/Precautions

Warning

Syncope and "First-dose" Effect

Terazosin hydrochloride tablets, like other alpha-adrenergic blocking agents, can cause marked lowering of blood pressure, especially postural hypotension, and syncope in association with the first dose or first few days of therapy. A similar effect can be anticipated if therapy is interrupted for several days and then restarted.

Syncope has also been reported with other alpha-adrenergic blocking agents in association with rapid dosage increases or the introduction of another antihypertensive drug. Syncope is believed to be due to an excessive postural hypotensive effect, although occasionally the syncopal episode has been preceded by a bout of severe supraventricular tachycardia with heart rates of 120-160 beats per minute. Additionally, the possibility of the contribution of hemodilution to the symptoms of postural hypotension should be considered.

To decrease the likelihood of syncope or excessive hypotension, treatment should always be initiated with a 1 mg dose of terazosin, given at bedtime. The 2 mg, 5 mg and 10 mg tablets are not indicated as initial therapy. Dosage should then be increased slowly, according to recommendations in the DOSAGE AND ADMINISTRATION and additional antihypertensive agents should be added with caution. The patient should be cautioned to avoid situations, such as driving or hazardous tasks, where injury could result should syncope occur during initiation of therapy.

If syncope occurs, the patient should be placed in a recumbent position and treated supportively as necessary. There is evidence that the orthostatic effect of terazosin is greater, even in chronic use, shortly after dosing. The risk of the events is greatest during the initial seven days of treatment, but continues at all time intervals.

Precaution

General

Prostatic Cancer: Carcinoma of the prostate and BPH cause many of the same symptoms. These two diseases frequently co-exist. Therefore, patient thought to have BPH should be examined prior to starting terazosin hydrochloride therapy to rule out the presence of carcinoma of the prostate.

Orthostatic Hypotension: While syncope is the most severe orthostatic effect of terazosin, other symptoms of lowered blood pressure, such as dizziness, lightheadedness and palpitations, were more common and occurred in some 28% of patients in clinical trials for hypertension. In BPH clinical trial, 21% of the patients experienced one or more of the following: dizziness, hypotension, Postural hypotension, syncope and vertigo. Patients with occupations in which such events represent potential problems should be treated with particular caution.

Pediatric Use

Safety and effectiveness in children have not been determined

Contraindications

Terazosin hydrochloride tablet are contraindicated in patients known to be hypersensitive to terazosin hydrochloride. It is not recommended for the treatment of heart failure caused by mechanical obstruction, i.e. Aortic or mitral valve stenosis, pulmonary embolism & restrictive pericardial disease.

Dosage and directions for use

Conmy Tablets 2mg is to be taken orally.

If Conmy administration is discontinued for several days, therapy should be reinstituted using the initial dosing regimen.

Benign Prostatic Hyperplasia:

Initial Dose: 1 mg at bedtime is the starting dose for all patients and this dose should not be exceeded. Patients should be closely followed during initial administration in order to minimize the risk of severe hypotensive response.

Subsequent Doses: The dose should be increased in a stepwise fashion to 2, 5 or 10 mg once daily to achieve the desired improvement of symptoms and/or flow rates. Doses of 10 mg once daily are generally required for the clinical response. Therefore, treatment with 10 mg for a minimum of 4-6 weeks may be required to assess whether a beneficial response has been achieved. Some patients may not achieve a clinical response despite appropriate titration. Although some additional patients responded to a 20-mg daily dose, there was an insufficient number of patients studied to draw definitive conclusions about this dose. There are insufficient data to support the use of higher doses for those patients who show inadequate or no response to 20 mg daily.

Use with Other Drugs: Caution should be observed when Conmy tablets are administered concomitantly with other hypertensive agents, especially the calcium channel blocker verapamil, to avoid the possibility of developing significant hypotension. When using Conmy tablets and other antihypertensive agents concomitantly, dosage reduction and retitration of either agent may be necessary.

Hypertension:

The dose of Conmy and the dose interval (12 or 24 hrs) should be adjusted according to the patient's individual blood pressure response. The following is a guide to its administration: Initial Dose: 1 mg at bedtime is the starting dose for all patients and this dose should not be exceeded. This initial dosing regimen should be strictly observed to minimize the potential for severe hypotensive effects.

Subsequent Doses: The dose may be slowly increased to achieve the desired blood pressure response. The usual recommended dose range is 1-5 mg once a day; however, some patients may benefit from doses as high as 20 mg/day. Doses >20 mg do not appear to provide further blood pressure effect and doses >40 mg have not been studied. Blood pressure should be monitored at the end of the dosing interval to be sure control is maintained throughout the interval. It may also be helpful to measure blood pressure 2-3 hrs after dosing to see if the maximum and minimum responses are similar, and to evaluate symptoms, eg dizziness or palpitations which can result from excessive hypotensive response. If response is substantially diminished at 24 hrs, an increased dose or use of a twice-daily regimen can be considered. If terazosin administration is discontinued for several days or longer, therapy should be reinstituted using the initial dosing regimen. In clinical trials, except for the initial dose, the dose was given in the morning.

Use in pregnancy and lactation

Pregnancy

Teratogenic Effect: Terazosin was not teratogenic in either rats or rabbits when administered oral doses up to 280 to 60 times; respectively, the maximum recommended human dose. Fetal resorptions occurred in rats with 480 mg/kg/day, approximately 280 times to maximum recommended human dose. Increased fetal resorptions, decreases fetal weight and an increased number of supernumerary ribs were observed in offspring of rabbits dosed with 60 times the maximum recommended human dose. These finding (in both species) were most likely secondary to maternal toxicity. There are no adequate and well-controlled studies in pregnant women and the safety of terazosin in pregnancy has not been established. Terazosin Hydrochloride is recommended during pregnancy unless the potential benefit justifies the potentials risk to the mother and fetus.

Non-teratogenic Effects : In a peri and post-natal development study in rats, significantly more pups died in the group dosed with 120mg/kg/day (>75 times the maximum recommended human dose) then in the control group during the three week postpartum period.

Nursing Mothers

It is not know whether terazosin is excreted in breast milk. Because many drugs are excreted in milk, caution should exercised when terazosin is administered to a nursing woman.

Effects on ability to drive & use machine

Dizziness, light-headedness or drowsiness may occur with the initial dose or in association with missed doses and subsequent reinitiation of terazosin therapy. Patients should be cautioned about these possible adverse events and the circumstances in which they may occur and advised to avoid driving or hazardous tasks for approximately the first 12 hours after the initial dose or when the dose is increased.

Drug interactions

In controlled trials, terazosin has been added to diuretics, and several beta-adrenergic blockers; no unexpected interactions were observed. Terazosin has also been used in patients on a variety of concomitant therapies; while these were not formal interaction studies, no interactions were observed. Terazosin has been used concomitantly in at least 50 patients on the following drugs or drug classes:

- 1) analgesic/anti-inflammatory (e.g., acetaminophen, aspirin, codeine, ibuprofen, indomethacin);
- 2) antibiotics (e.g., erythromycin, trimethoprim and sulfamethoxazole);
- 3) anticholinergic/sympathomimetics (e.g., phenylephrine hydrochloride, phenylpropanolamine hydrochloride, pseudoephedrine hydrochloride);
- 4) antigout (e.g., allopurinol);
- 5) antihistamines (e.g., chlorpheniramine);
- 6) cardiovascular agents (e.g., atenolol, hydrochlorothiazide, methyclothiazide, propranolol);
- 7) corticosteroids;
- 8) gastrointestinal agents (e.g., antacids);
- 9) hypoglycemics;
- 10) sedatives and tranquilizers (e.g., diazepam).

Use with Other Drugs: caution should be observed when terazosin hydrochloride is administered concomitantly with other antihypertensive agents, especially the calcium channel blocker verapamil to avoid the possibility of developing significant hypotension. When using terazosin hydrochloride and other antihypertensive agents concomitantly, dosage reduction and either agent may be necessary.

Overdosage, symptoms and treatment

Should overdosage of terazosin hydrochloride lead to hypotension, support of the cardiovascular system is of first importance. Restoration of blood pressure and normalization of heart rate may be accomplished by keeping the patient in the supine position. If this measure is inadequate, shock should first be treated with volume expanders. If necessary, vasopressors should then be used and renal function should be monitored and supported as needed. Laboratory data indicate that terazosin is 90-94% protein bound; therefore, dialysis may not be of benefit.

Storage conditions

Store at temperature below 30°C. Protect from light. Keep out of reach of children.

Shelf-life

36 months from the date of manufacture if kept as recommended.

Package

10 tablets x 1 piece (PTP Package), 3 pieces per box and 10 pieces per box.

Registration number

CONMY TABLETS 2mg – MAL09100245AZ

Manufacturer

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Product Registration Holder

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